

[illegible]

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
المقر بما فيه
التوقيع: (-----) التوقيع: (-----)

خطه العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي
الاورام المنتشر [14456]
التعليق على التشخيص : -----

الاسم: د/محمد كيلاني
 الاسم: د/ندي حسن
 الاسم: د/نرمين رمضان
 الوظيفة: _____
 الوظيفة: _____
 الوظيفة: _____
 التوقيع: _____
 التوقيع: _____
 التوقيع: _____
 تعتمد صحة البيانات المرفقة
 رئيس اللجنة ومدير
 المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د.ا ----- / د.ا

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 جهة العلاج :
 نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

Name: امال جابر حسين سليمان
National ID: 25711182101506



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Pin Medication And Anti Emetics

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

Patient's Name: Amal Jaber Hussein
Date: 2026-06-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS: -

Compared to attached study report dated 15-11-2025, the current study shows:

I Chest:

- Mildly enlarged size regarding the previously seen left breast lesions largest measuring 3.7 X 2.7 cm in today's study, as well as the left axillary lymph nodes the largest measuring 2.1 x 1.8 cm in today's study.
- Rather stationary course regarding the previously seen left supraclavicular lymph nodes.
- Progressive course regarding the size and number of the previously noted multiple bilateral scattered pulmonary nodules largest measures 12.7 x 10 mm at the left lower lung lobe.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral elastofibroma dorsi more sizable on the left side.

II Abdomen & pelvis:

- Enlarged liver with homogenous low CT texture. Still no focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The examined skeleton revealed:
 - Newly developed manubrium sterni lytic bone lesion.

Patient's Name: Amal Jaber Hussein
Date: 2026-06-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- Stationary course regarding the previously seen left iliac heterogenous mixed lytic and sclerotic lesion.
- Still noted healing sternal fracture.
- Left hip osteoarthritic changes with geodes.
- Dorsolumbar spondylosis and scoliosis.

Opinion:

- Progressive course regarding the previously seen left breast lesion, left axillary lymph nodes, pulmonary nodules as well as the newly developed manubrium sterni lytic bone lesion.
- Stationary course regarding the rest of the study.

Dr.sherif Wagdy

Dr.sherif Wagdy

04/06/2026

MDT FORM

Name:	Amal Jaber Huseen .	MRN:	363
Age:	68 Y	ID:	25711182101506
Menopausal Sta	postm	Phone:	01129998588
NRS SCORE:	ps 1	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: Idc
ER: 8/8 PR: 4/8 HER2: 3+ KI67:

R

L

Sonomammography:

Mets Workup:

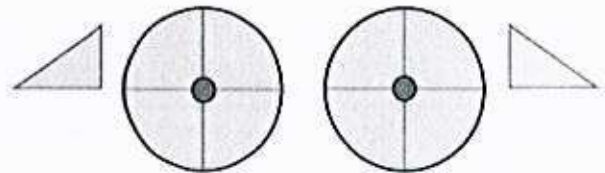
History:

68 Y POSTM LEFT BREAST CANCER MET TO BONE ER8/8 PR4/8 HER23+
DIAGNOSED IN 2022 ST FEMARA AND HERCEPTIN TILL 1/2025 PD ST XELODA
TYKERB .CTS 11/2025 INCREASE SIZE OF PUL NODULE 9 MM VS PREVIOUS 2
TO 3 MM bone scan on 9/3/2026 comp to bs 4/2025 from 3/2025 mild
progression of metastatic bone deposits ,sd of osseous lesions with newly
developped osseous lesion at sternum and left 6th rib for mdt(pt st xeloda
tykerb from 3/2025 and tykerb stopped for few months bec of approval)

Recommendations:

MDT Outcome & Plan

for navelbine + - tykerb (depending on approval)



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr.henan ezzat					

جمهورية مصر العربية
بطاقة تحقيق الشخصية

أمال
جابر حسين سليمان
١٨ محمد صبحي المحامي - أرض اللواء
العجوزة - الجيزة

٢٥٧١١١٨٢١.١٥.٦

KJ9459820

٢٠٢٤/٠٢ ٢٥٧١١١٨٢١.١٥.٦

رربة منزل

أنثى مسجلة أرملة

البطاقة سارية حتى ٢٠٣١/٠٢/٠٦

١



SCAN 8 LABS

Name : Amal Jaber Hussein Suleiman

Branch : Dokki

Ref.Physician Prof : Goustav Roussey Egypt

مركز النيل

Date : 09-03-2026 08:37

Medical No. : 329545

Ins.Company : تعاقد مستشفيات

ISOTOPIC BONE SCAN REPORT

- **Indication:** 68-year-old female with history of metastatic breast carcinoma; follow-up assessment is requested. Prior bone scan dated 15/04/2025 demonstrated few osseous lesions, likely metastatic in nature.
- **Radioisotope:** Tc-99m MDP
- **Imaging Protocol:** Whole-body planar images (anterior and posterior projections) acquired approximately 2–3 hours post intravenous injection of the tracer

FINDINGS:

In comparison with the prior bone scan dated 15/04/2025, the current study demonstrates:

- **Mildly regressed** tracer uptake intensity at the known osseous lesion involving the **left acetabulum**.
- **Stationary course** of the known osseous lesions at the posterior aspect of the **left 7th rib** and anterior aspect of the **right 4th rib**.
- **Newly detected osseous lesions** at the **sternum** and anterior aspect of the **left 6th rib**, indicating disease progression.
- Degenerative/arthritis changes are noted at multiple levels of the spine (with associated lumbar scoliosis), both shoulders, and the weight-bearing joints.

IMPRESSION:

- **Mild progression of metastatic bone disease** since the prior study dated 15/04/2025 as above-detailed.
- Clinical and imaging follow-up is recommended as clinically indicated.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

A-Patient ID:
1- Name : **امان جابر حسين شفيان**

Sex:

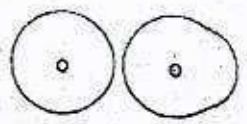
MDT form
3- Medical number:
Female

83047945
Old Med :

Old Med :
5380-2021

2-ID number:

Patient Age : 67 Y



Family History

Phone

01142430699

BMI

marital status

NRS Score

GROUP

Co morbidity

Menopausal status

Initial STAGE

REASON OF MDT

REASON OF MDT

Date of MDT

16/02/2025

Breast Imaging :

Clinical data

Mets Workup :

BX IDC	ER 8/8	PR 4/8	Her 2 Neu	SISH	KI67
COMMENT	COMMENT	COMMENT	COMMENT	COMMENT	

T	N	M
		1 bone

Sonomammography

PATIENT HISTORY

MBC to bone
pt received herceptin iv x 27 cycle+AI last 11/2023
kept under AI+zometa till now
12/2024 CTCAP , BSM : progressive breast lesion , stationary bone mets
bone_scan requested & pending

MDT OUTCOME & PLAN

for 2nd line Xeloda / tykerb x 3 mns then reass.

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY Signature: <i>[Signature]</i>	RADIOLOGY	RADIOLOGY Signature: <i>[Signature]</i>
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Patient Name : الأستاذة / امال جابر حسين سليمان
Patient No. : 25711182101506
Path No. : SR-743/21
Sex : Female
Age : 64 Y
Referring Doc :
Request No. : 133527
Request Date : 20/12/2021
Verification Date : 28/12/2021
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

Clinical Data:

Left breast mass

Nature of the specimen:

Slide revision (Tru-Cut biopsy)

Gross Examination:

1 block (9332/21) & 3 slides

Microscopic examination:

- Histological type: Invasive duct carcinoma grade II
- Angioinvasion: No
- Ductal carcinoma in situ: No
- Lymph node involvement: N/A
- Resection Margins: N/A
- Histologic response to therapy: N/A

Diagnosis:

Left breast mass; Slide revision:

Invasive duct carcinoma grade II

IHC:

ER: Strong nuclear reaction in 97% of tumour cells. Positive, score (8/8)

PR: Moderate nuclear reaction in 20% of tumour cells. Positive, score (4/8)

Her-2: Strong complete membrane reaction in 10% of tumour cells. Positive, score 3+

Prof. Dr. Amal Hareedy

Dr. Abdulrahman Ibrahim

Verified by :

abdelrahman ibrahim

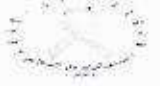
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Page 1 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name الأستاذة / امال جابر حسين سليمان
Patient No. 25711182101506
Path No. SR-743/21
Sex Female
Age 64 Y
Referring Doc.

Request No. 133527
Request Date 20/12/2021
Verification Date 28/12/2021
Financial Category اورام دار السلام - نفقة الدولة
Department Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Verified by :
abdelrahman ibrahim

Printing Date : 11/02/2025 10:13:20

Page 2 of 2

Printed By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



SCAN & LABS

مركز النيل

Name : Amal Jaber Hussein Suleiman

Date : 09-03-2026 08:37

Branch : Dokki

Medical No. : 329545

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعافد مستشفيات

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IMPRESSION:

- **Mild progression of metastatic bone disease** since the prior study dated 15/04/2025 as above-detailed.
- Clinical and imaging follow-up is recommended as clinically indicated.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine



Patient Name: Amaal Gaber Hussein Slimin	Date : 11/02/2025
Exam : MAMMOGRAPHY BOTH	Age : 67 Year(s)
Medical ID: 5386/2021	Pat. ID: 18620

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Patient is 67 years old and is seen for metastatic left breast malignant mass

Report:

- ACR c: heterogeneous fibro-glandular parenchyma of both breasts
- Left Breast:
 - A dense spiculated irregular mass is seen in the left UOQ measuring 45x40x35mm about 40mm from the nipple and attached to the overlying skin.
 - Ultrasound shows multiple smaller suspicious masses in the UOQ and in the lower inner quadrant at 7O'clock zone c measuring 9x8mm.
 - Multiple (>3) pathologically enlarged level I axillary lymph nodes. The largest is measuring 27x16mm.
 - No levels II or III axillary or supraclavicular LNS
- Right Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - A well-defined oval parallel hypoechoic mass is seen at 10'clock measuring about 11x7mm.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Metastatic left breast malignant mass with multicentric disease and ipsilateral axillary lymphadenopathy (BIRADS 6)
- Probably benign findings of right breast (BIRADS 3)

Best regards,
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
11/02/2025 1:15 PM



Total DLP (mGy.cm):

Patient Name: Amaal Gaber Hussein Slimin	Date : 20/01/2024
Exam : C.T CHEST WITH CONTRAST	Age : 66 Year(s)
Medical ID: 5386/2021	

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS: -**

**Compared to attached study report dated 24/01/2023, the
current study shows:**

I Chest:

- Stationary course regarding the left breast lesions largest meaasures 1.6 x 1.8 cm and left axillary level I & II subcentemtric lymph nodes.
- No sizabel pulmonary nodules, masses or consolidations.
- Still noted bilateral basal ground glass faint attenuations.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral elastofibroma dorsi more sizable on the left side.

II Abdomen & pelvis:

- Enlarged liver with homogenous low CT texture. Still no focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.



Total DLP (mGy.cm):

Patient Name: Amaal Gaber Hussein Slimin	Date : 20/01/2024
Exam : C.T CHEST WITH CONTRAST	Age : 66 Year(s)
Medical ID: 5386/2021	

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The examined skeleton revealed:
 - Stationary left iliac lytic lesion heterogenous sclerotic lesion.
 - Left hip osteoarthritic changes with geodes.
 - Dorsolumbar spondylosis and scoliosis.

Opinion:

- Follow up study showing overall stationary course.

Dr.Shaimaa Megahed,MSc

Dr. Ofa Magdy, M.D.

Kasr El-Ainy Center of Radiation Oncology and
Nuclear Medicine

Faculty Of Medicine, Cairo University

"NEMROCK"

Nuclear Medicine Unit



مركز قصر العيني

لعلاج الأورام والطب النووي
كلية الطب - جامعة القاهرة
(ممرات)

Report

Patient Name:	Amal Gaber Hussein Soliman	Patient Gender:	Female
Patient Code:	Q44547	DATE:	4/15/2025 10:31:58 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- o 68-year-old female patient with history of left breast cancer.

Technique

- o Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- o Active tracer uptake is noted in the left acetabulum, right 4th rib anteriorly and left 10th rib posteriorly.
- o Arthritic changes of the knees and shoulders are noted.
- o Patchy tracer uptake is noted in the spine more pronounced in lumbar region? Degenerative? Metastatic, for MRI confirmation to justify its nature.
- o No other serious bone lesions detected.

Opinion

- o Few osseous lesions as described likely metastatic in nature.

Dr. Hagar Nageh

Dr. Nahla Dessoki, MD

